

HCOS™ Healthcare Instruments

Human-Guided Tools for Understanding, Reflection, and System Improvement

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Issuance Statement

This document is issued as **Version 1.1 — Draft** for public learning, review, testing, research, and continued development within the HCOS™ Healthcare Systems discipline.

It introduces the purpose, structure, safeguards, and proposed applications of HCOS™ Healthcare Instruments.

This document does not establish a clinical practice guideline, regulatory requirement, certification standard, or independent medical decision-making system.

HCOS™ Healthcare Instruments are intended to support human understanding and responsible collaboration.

They do not replace professional judgment, clinical expertise, ethical responsibility, appropriate governance, or meaningful human relationships.

This document may be revised as new evidence, practical experience, community feedback, and validation findings become available.

Overview

HCOS™ Healthcare Instruments are human-guided tools designed to support:

- Reflection
- Learning
- Systems analysis
- Communication
- Structured review
- Responsible decision-making
- Healthcare improvement

Some instruments may use artificial intelligence.

However, an HCOS™ Instrument is not an independent authority, autonomous decision-maker, or replacement for human judgment.

The instrument supports the work.

The human remains responsible for understanding, interpretation, verification, decision-making, and action.

Why HCOS™ Uses the Term “Instrument”

An instrument helps a person observe, examine, measure, organize, or understand something more clearly.

A stethoscope can help a healthcare professional hear information that may otherwise be difficult to detect.

A map can help someone understand a path.

A checklist can help people remember important steps.

None of these tools independently understands the complete situation or determines what should happen next.

Their value depends on how people use them.

In the same way, an HCOS™ Instrument may help people identify:

- Patterns
- Pressures
- Relationships
- Risks
- Missing information
- Unclear responsibilities
- Possible system barriers
- Opportunities for improvement

Its purpose is to extend human understanding—not replace human responsibility.

Definition

An HCOS™ Instrument is a human-guided tool that supports understanding, reflection, analysis, communication, or improvement. It may be AI-enabled, but it requires human collaboration, contextual interpretation, and accountable human judgment.

Human–Instrument Collaboration

HCOS™ Instruments are designed around collaboration between the person using the instrument and the technology, framework, worksheet, or process supporting the review.

The Instrument May Contribute

An instrument may help:

- Organize information
- Apply a structured framework
- Ask consistent questions
- Identify possible patterns
- Reveal pressure points
- Compare conditions with HCOS™ principles
- Translate complex information
- Document observations
- Identify missing perspectives
- Suggest areas requiring deeper review

The Human Contributes

The person using the instrument contributes:

- Lived experience
- Professional knowledge
- Organizational context
- Understanding of relationships
- Ethical judgment
- Compassion
- Interpretation
- Verification
- Accountability
- Decision-making authority
- Awareness of circumstances the instrument cannot fully know

The instrument may generate information or possible interpretations.

The human determines whether those interpretations are accurate, appropriate, ethical, and useful.

Collaborative Use Process

1. The Human Defines the Concern

The person identifies the situation, question, experience, workflow, policy, or system that needs to be examined.

The concern should be described as clearly as possible without assuming that the visible problem is the complete problem.

2. The Instrument Supports Structured Exploration

The instrument organizes available information and applies relevant HCOS™ questions, principles, or frameworks.

It may help reveal:

- Who is affected
 - Where pressure exists
 - How work moves
 - Where responsibility is unclear
 - Which protections may be missing
 - What additional information is needed
-

3. The Human Adds Context

The person evaluates the output using:

- Lived experience

- Professional expertise
 - Patient and caregiver perspectives
 - Organizational knowledge
 - Scientific evidence
 - Ethical responsibilities
 - Legal and regulatory requirements
 - Understanding of the local environment
-

4. The Human Reviews for Accuracy and Harm

The person reviews the output for:

- Incorrect information
 - Unsupported assumptions
 - Missing perspectives
 - Bias
 - Oversimplification
 - False certainty
 - Unequal effects
 - Potential unintended consequences
 - Recommendations that exceed the instrument's purpose
-

5. Humans Determine the Appropriate Action

Decisions remain with the people who possess the relevant:

- Responsibility
- Authority
- Knowledge
- Professional qualifications
- Relationship to the people affected

An instrument may support a decision.

It should not hide who is responsible for making or approving that decision.

6. The System Continues Learning

After the instrument is used, people should examine:

- What was learned
- What remained unclear
- Whether the output was useful
- What action was taken
- What changed
- What unintended effects occurred

- How the instrument or process should improve
-

Proposed HCOS™ Healthcare Instruments

HCOS™ Healthcare Systems Instrument

Helps people examine a healthcare concern using:

- The HCOS™ Journey
- The eight system forces
- The three human protections
- The meaning anchor of love

The instrument may help users identify the people affected, pressure pathways, system conditions, tradeoffs, missing protections, and possible areas for improvement.

Human Load Check Instrument

Helps individuals and teams recognize:

- Workload concentration
- Invisible labor
- Cognitive fragmentation
- Responsibility without adequate authority
- Recovery gaps
- Dependence on personal sacrifice
- Early signs of system overload

The instrument does not diagnose burnout or determine an individual's personal capacity.

It helps people begin a structured conversation about whether the surrounding system is sustainable.

Annotated evidence connection: The instrument's occupational-burnout boundary is informed by the World Health Organization's ICD-11 classification of burn-out as an occupational phenomenon (World Health Organization, 2019). Its attention to exhaustion, distancing, efficacy, and workplace conditions is informed by Maslach and Jackson (1981) and Maslach and Leiter (2016). These sources inform the instrument's conceptual design; they do not validate the instrument or make it a diagnostic measure.

Healthcare Workflow Review Instrument

Helps people map how work, information, decisions, and responsibility move through a healthcare process.

It may help identify:

- Delays
 - Repeated work
 - Unclear ownership
 - Missing information
 - Unsafe handoffs
 - Manual coordination
 - Workarounds
 - Tasks dependent on individual memory
 - Points where people compensate for system design
-

Access Pathway Review Instrument

Helps people examine whether patients and families can realistically enter, navigate, and continue through a healthcare pathway.

It may explore barriers involving:

- Scheduling
 - Referrals
 - Transportation
 - Cost
 - Insurance coverage
 - Language
 - Technology
 - Health literacy
 - Disability
 - Geography
 - Caregiving responsibilities
 - Coordination among organizations
-

Healthcare Communication Instrument

Helps people make healthcare communication clearer, more respectful, and easier to understand.

It may support:

- Patient education
- Care instructions
- Professional communication
- Policy explanations
- Consent materials
- Leadership communication

- Public-facing healthcare information

All final communication requires human review for accuracy, meaning, tone, safety, and appropriateness for the intended audience.

Annotated evidence connection: Edmondson (1999) supports the importance of interpersonal conditions that allow people to ask questions, seek help, discuss errors, and raise concerns. Maslach and Leiter (2017) connect the social environment and workplace civility with burnout prevention. These sources support respectful communication and voice as protective conditions, but they do not establish that communication or civility can substitute for safe staffing, manageable workload, adequate resources, fair procedures, or organizational action.

Policy and Metric Pressure Instrument

Helps people examine how policies and measurements influence:

- Behavior
- Workload
- Access
- Decision-making
- Resource use
- Patient care
- Workforce sustainability

Annotated evidence connection: Maslach and Leiter (2016) identify workload, control, reward, community, fairness, and values as six areas of worklife in which persistent mismatches may contribute to burnout. This framework supports review of how policies and metrics shape workload, agency, recognition, relationships, fairness, and ethical alignment. It does not prove that any single policy or metric caused burnout in a particular person.

The instrument may ask:

- What is the intended purpose?
 - What work does this create?
 - Who performs that work?
 - What behavior does the measure encourage?
 - What meaningful work may be overlooked?
 - Who may be helped, burdened, or excluded?
 - What unintended consequences should be monitored?
-

Healthcare Technology and AI Review Instrument

Helps organizations examine a proposed or existing technology through a human-centered systems lens.

It may support review of:

- The human problem being addressed
- Existing workflow conditions
- Data quality
- Human oversight
- Accountability
- Privacy
- Security
- Equity
- Validation
- Failure modes
- New work created by the technology
- Effects on patients and care teams
- Monitoring requirements
- Escalation pathways
- Stopping conditions

The instrument supports review.

It does not independently authorize a technology for clinical use.

Annotated evidence connection: Edmondson (1999) supports the need for conditions in which people can report errors, uncertainty, and concerns without excessive interpersonal risk. Applied to healthcare technology and AI, HCOS™ treats voice, contestability, escalation, and visible accountability as important safeguards. This is an HCOS™ application of the psychological-safety literature; Edmondson's original study did not evaluate artificial intelligence or clinical technology governance.

Annotated Evidence Instrument

Helps people organize evidence and consider how research may inform:

- HCOS™ principles
- Frameworks
- Standards
- Methods
- Instruments
- Learning resources
- Future research

The instrument should distinguish among:

- Established findings
- Emerging evidence
- Study limitations
- Author interpretation
- HCOS™ synthesis
- Proposed application

- Unanswered questions

Annotated evidence connection: The attached evidence appendix demonstrates this separation. It distinguishes authoritative classification from empirical research, measurement from causation, direct findings from HCOS™ interpretation, and conceptual relevance from instrument validation.

Instrument Requirements

Every HCOS™ Healthcare Instrument should:

- Have a clearly defined purpose
 - Identify its intended users
 - Explain what it can and cannot do
 - Require meaningful human participation
 - Preserve human judgment
 - Preserve identifiable human accountability
 - Protect privacy and confidentiality
 - Use appropriate and traceable evidence
 - Distinguish evidence from interpretation
 - Identify uncertainty and missing information
 - Avoid presenting possibilities as confirmed conclusions
 - Include safeguards against bias and unequal effects
 - Make escalation pathways clear
 - Be reviewed for unintended consequences
 - Be revised as evidence and experience develop
 - Reduce rather than transfer unnecessary human burden
 - Support human capability without diminishing human value
-

Instrument Documentation Requirements

Each individual HCOS™ Healthcare Instrument should include:

Identification

- Instrument title
- Instrument code
- Version
- Status
- Issue date
- Issuing organization
- Document steward

- Repository file name
- Repository location

Purpose

Explain the human or system need the instrument is intended to address.

Intended Users

Identify who may appropriately use the instrument.

Appropriate Uses

Describe the circumstances in which the instrument may be helpful.

Inappropriate Uses

Identify situations in which the instrument should not be used.

Required Human Collaboration

Explain what human knowledge, interpretation, verification, or authority is required.

Instructions

Provide clear steps for using the instrument.

Inputs

Identify the information required to use the instrument responsibly.

Outputs

Explain what the instrument produces and how the output should be interpreted.

Limitations

Describe what the instrument cannot know, determine, or guarantee.

Evidence

Identify the evidence, frameworks, and standards supporting the instrument.

Evidence documentation should distinguish:

- What the cited source directly established
- What the authors interpreted
- What HCOS™ infers or proposes

- What remains uncertain
- Whether the source supports the instrument's concept, content, implementation, or validation
- Whether permission or licensing is required for any incorporated measure

Safeguards

Describe protections involving privacy, bias, validation, escalation, oversight, and accountability.

Review and Revision

Explain how feedback, evidence, incidents, and practical experience will inform future versions.

Instruments Are Not Independent Decision-Makers

Unless separately developed, evaluated, validated, approved, and governed for a specific clinical purpose, an HCOS™ Healthcare Instrument should not be used to:

- Diagnose a health condition
- Select or prescribe treatment
- Replace emergency guidance
- Override professional judgment
- Determine an individual's rights
- Determine access to healthcare
- Make autonomous clinical decisions
- Replace informed consent
- Replace communication between patients and healthcare professionals
- Serve as the sole basis for employment decisions
- Serve as the sole basis for disciplinary action
- Serve as the sole basis for safety determinations
- Serve as the sole basis for resource-allocation decisions

An HCOS™ Instrument may help people think more clearly.

It does not remove the need for people to:

- Think
- Listen
- Verify
- Communicate
- Exercise judgment
- Accept responsibility
- Remain present
- Care for one another

Relationship to HCOS™ Healthcare Systems

This document is part of:

D-003 — HCOS™ Healthcare Systems: A Framework for Human-Centered Healthcare Practice

The discipline provides the broader philosophy, principles, framework, journey, standards, methods, research foundation, learning resources, and future direction for human-centered healthcare practice.

The instruments described here translate that discipline into structured tools people can use collaboratively.

Repository Structure

```
D-003-Healthcare-Systems/  
├── 08-instruments/  
│   ├── README.md  
│   ├── healthcare-systems-instrument/  
│   │   ├── README.md  
│   │   ├── human-load-check-instrument/  
│   │   │   ├── README.md  
│   │   ├── healthcare-workflow-review-instrument/  
│   │   │   ├── README.md  
│   │   ├── access-pathway-review-instrument/  
│   │   │   ├── README.md  
│   │   ├── healthcare-communication-instrument/  
│   │   │   ├── README.md  
│   │   ├── policy-and-metric-pressure-instrument/  
│   │   │   ├── README.md  
│   │   ├── healthcare-technology-and-ai-review-instrument/  
│   │   │   ├── README.md  
│   └── annotated-evidence-instrument/  
│       ├── README.md
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Appendix A — Annotated Evidence Foundation

Purpose of This Appendix

This appendix records the principal evidence sources currently connected to the design of the HCOS™ Healthcare Instruments described in this document.

The presence of a source in this appendix means that it informs a concept, boundary, safeguard, or design question. It does **not** mean that the source validates an HCOS™ instrument, establishes clinical effectiveness, or authorizes independent decision-making.

Each annotation separates:

- The source's direct contribution
- Its relevance to HCOS™ Healthcare Instruments
- Its principal evidence boundary

Evidence-to-Instrument Traceability

Evidence source	Direct contribution	Primary instrument connections	Key boundary
World Health Organization (2019)	Defines burn-out as an occupational phenomenon associated with chronic workplace stress that has not been successfully managed	Human Load Check Instrument; Healthcare Systems Instrument	Classification source, not an intervention or instrument-validation study
Maslach and Jackson (1981)	Establishes the multidimensional measurement structure underlying the Maslach Burnout Inventory	Human Load Check Instrument; Annotated Evidence Instrument	Measures burnout dimensions; does not identify system causes or provide a clinical diagnosis
Maslach and Leiter (2016)	Synthesizes burnout as exhaustion, cynicism, and reduced efficacy and identifies six areas of worklife	Human Load Check; Policy and Metric Pressure; Healthcare Systems Instrument	Narrative synthesis; does not prove that a specific mismatch caused an individual outcome
Edmondson (1999)	Defines team psychological safety and links it with team learning behaviors	Healthcare Communication; Healthcare Technology and AI Review; Healthcare Systems Instrument	Did not study burnout, healthcare, or AI directly
Maslach and Leiter (2017)	Reviews the importance of the healthcare social environment, civility, and organizational approaches to burnout	Healthcare Communication; Human Load Check; Healthcare Systems Instrument	Civility should complement, not replace, structural reform

AE-HC-001 — Burn-out as an Occupational Phenomenon

Citation

World Health Organization. (2019, May 28). *Burn-out an “occupational phenomenon”: International Classification of Diseases*. <https://www.who.int/news/item/28-05-2019-burn-out-an-occupational-phenomenon-international-classification-of-diseases>

Source Type

Authoritative international classification statement.

Direct Contribution

The World Health Organization describes burn-out in ICD-11 as a syndrome conceptualized as resulting from chronic workplace stress that has not been successfully managed. It identifies three dimensions:

1. Energy depletion or exhaustion
2. Increased mental distance, negativism, or cynicism related to work
3. Reduced professional efficacy

WHO classifies burn-out as an occupational phenomenon rather than a medical condition and limits the construct to the occupational context. WHO also clarifies that burn-out was present in ICD-10 within the same broad category, while ICD-11 provides a more detailed definition.

HCOS™ Instrument Relevance

This source supports the Human Load Check Instrument's boundary against diagnosing a medical condition. It also supports directing attention toward the occupational environment when chronic workplace stress remains unresolved.

Within HCOS™, “not successfully managed” should not automatically be interpreted as individual failure. Occupational stress management may depend on workload, workflow, resources, authority, leadership decisions, recovery opportunities, and wider organizational conditions.

Evidence Boundary

This classification does not identify the cause of burn-out in a particular person, prescribe a measurement tool, establish diagnostic thresholds, or prove that a particular intervention will work. It also does not exclude the possibility that a person may have a coexisting medical or mental health condition requiring appropriate assessment.

AE-HC-002 — Measurement of Experienced Burnout

Citation

Maslach, C., & Jackson, S. E. (1981). The measurement of experienced burnout. *Journal of Occupational Behaviour*, 2(2), 99–113. <https://doi.org/10.1002/job.4030020205>

Source Type

Psychometric instrument-development study.

Direct Contribution

Maslach and Jackson (1981) developed and evaluated a multidimensional measure of experienced burnout. The work supported three related dimensions commonly described as emotional exhaustion, depersonalization, and personal accomplishment.

HCOS™ Instrument Relevance

The article helps distinguish a human outcome measure from a system diagnostic. Within HCOS™:

- Emotional exhaustion may indicate depletion.
- Depersonalization may reflect relational distancing or defensive adaptation.
- Reduced personal accomplishment may reflect blocked efficacy or diminished ability to experience meaningful contribution.

These are interpretive possibilities, not automatic conclusions about cause.

Evidence Boundary

The Maslach Burnout Inventory does not independently diagnose a medical or psychiatric condition, identify which workplace factor caused a score, or determine the appropriate intervention. HCOS™ instruments should not reproduce proprietary inventory items or scoring instructions without appropriate authorization and licensing.

AE-HC-003 — Burnout and the Six Areas of Worklife

Citation

Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience: Recent research and its implications for psychiatry. *World Psychiatry, 15*(2), 103–111. <https://doi.org/10.1002/wps.20311>

Source Type

Narrative review and conceptual synthesis.

Direct Contribution

Maslach and Leiter (2016) describe burnout as an occupational syndrome involving exhaustion, cynicism, and reduced professional efficacy. They identify six areas of worklife in which persistent mismatches may contribute to burnout:

1. Workload
2. Control
3. Reward
4. Community
5. Fairness
6. Values

The authors emphasize changing workplace conditions rather than relying exclusively on individual coping strategies.

HCOS™ Instrument Relevance

The six areas provide an evidence-informed structure for examining:

- Whether demand exceeds sustainable capacity
- Whether responsibility is paired with meaningful authority
- Whether effort is recognized and reciprocated
- Whether relationships support trust and belonging
- Whether burdens and decisions are distributed fairly
- Whether organizational demands conflict with professional or ethical values

This source directly informs the Human Load Check Instrument and the Policy and Metric Pressure Instrument.

Evidence Boundary

The article does not establish that all six mismatches must be present, that any one mismatch caused burnout in a particular individual, or that individual support has no value. Workplace redesign and human support may both be necessary.

AE-HC-004 — Psychological Safety and Learning Behavior

Citation

Edmondson, A. C. (1999). Psychological safety and learning behavior in work teams. *Administrative Science Quarterly*, 44(2), 350–383. <https://doi.org/10.2307/2666999>

Source Type

Multimethod organizational field study.

Direct Contribution

Edmondson (1999) defines team psychological safety as a shared belief that the team is safe for interpersonal risk-taking. Psychological safety was associated with learning behaviors such as asking questions, seeking feedback, discussing errors, experimenting, and requesting help.

HCOS™ Instrument Relevance

This study supports designing instruments and implementation processes that make it easier for people to:

- Admit uncertainty
- Report errors
- Request assistance
- Challenge assumptions
- Raise concerns
- Participate in collective learning

For healthcare technology and AI, HCOS™ extends this logic to contestability, escalation, and the ability to question or override a technology-supported recommendation.

Evidence Boundary

The original study did not measure burnout and was not conducted as a healthcare or AI-governance study. Its application to burnout prevention, clinical environments, and AI oversight is a reasoned extension that requires context-specific evidence and evaluation.

Psychological safety also cannot compensate indefinitely for inadequate staffing, excessive workload, insufficient resources, or organizational unwillingness to act.

AE-HC-005 – Civility, Social Environment, and Burnout

Citation

Maslach, C., & Leiter, M. P. (2017). New insights into burnout and health care: Strategies for improving civility and alleviating burnout. *Medical Teacher*, 39(2), 160–163.
<https://doi.org/10.1080/0142159X.2016.1248918>

Source Type

Narrative review and conceptual overview focused on healthcare and medical education.

Direct Contribution

Maslach and Leiter emphasize the importance of the social environment in healthcare and discuss workplace civility as a potentially modifiable organizational condition. The article considers how respectful social encounters and civility-focused interventions may contribute to reductions in burnout.

HCOS™ Instrument Relevance

This source supports attention to relational human load within the Healthcare Communication Instrument and Healthcare Systems Instrument. Incivility can add vigilance, conflict-management burden, interpersonal anxiety, and reluctance to seek help.

Civility may strengthen community, trust, belonging, communication, and access to support.

Evidence Boundary

Civility should not be treated as a substitute for adequate staffing, manageable workload, fair procedures, sufficient resources, appropriate authority, or recovery time. A respectful culture can reduce relational harm, but it cannot make structurally unsafe work sustainable.

Cross-Evidence HCOS™ Synthesis

Together, these sources support a layered approach:

1. Recognize the Human Outcome

Burnout-related experiences may involve exhaustion, distancing or cynicism, and reduced efficacy.

2. Preserve the Occupational Boundary

Burn-out is specifically situated in the occupational context and should not be used as a universal label for distress in every area of life.

3. Investigate the Work System

Potential system domains include workload, control, reward, community, fairness, values, leadership decisions, workflow, resources, and recovery time.

4. Protect Voice and Learning

People need conditions in which they can ask for help, discuss errors, report pressure, and challenge unsafe assumptions.

5. Address Relational and Structural Conditions

Civility and psychological safety are meaningful protections, but they must be accompanied by action on structural sources of pressure.

6. Avoid Diagnostic Overreach

HCOS™ instruments may support recognition, reflection, and structured review. They do not diagnose burnout or another health condition.

7. Keep Evidence and Interpretation Separate

Instrument outputs should identify whether a statement represents:

- A direct research finding
- An author interpretation
- An HCOS™ synthesis
- A hypothesis requiring testing
- A proposed application
- An unresolved question

Appendix B – Bibliography

Edmondson, A. C. (1999). Psychological safety and learning behavior in work teams. *Administrative Science Quarterly*, 44(2), 350–383. <https://doi.org/10.2307/2666999>

Maslach, C., & Jackson, S. E. (1981). The measurement of experienced burnout. *Journal of Occupational Behaviour*, 2(2), 99–113. <https://doi.org/10.1002/job.4030020205>

Maslach, C., & Leiter, M. P. (2016). Understanding the burnout experience: Recent research and its implications for psychiatry. *World Psychiatry*, 15(2), 103–111. <https://doi.org/10.1002/wps.20311>

Maslach, C., & Leiter, M. P. (2017). New insights into burnout and health care: Strategies for improving civility and alleviating burnout. *Medical Teacher*, 39(2), 160–163.
<https://doi.org/10.1080/0142159X.2016.1248918>

World Health Organization. (2019, May 28). *Burn-out an “occupational phenomenon”: International Classification of Diseases*. <https://www.who.int/news/item/28-05-2019-burn-out-an-occupational-phenomenon-international-classification-of-diseases>

Version History

Version	Date	Status	Description
1.1	July 19, 2026	Draft	Added annotated evidence connections, evidence-to-instrument traceability, evidence boundaries, and bibliography.
1.0	July 19, 2026	Draft	Initial issuance of the HCOS™ Healthcare Instruments overview and human–instrument collaboration requirements.

Citation

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Closing Principle

The instrument supports the work. The human provides the context, judgment, compassion, and accountability.

HCOS™ Healthcare Instruments are intended to help people see more clearly without surrendering the responsibility to think, understand, protect, and care.

Think Well.

Teach Well.

Understand Well.

Grow Well.

Lead with Love.

See the Whole System.

Protect with Wisdom, Compassion, and Presence.

Help People Flourish.